

Flank Pain - Female (Adult)

Algorithm 1174 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only assessment of flank, side, or back pain where renal stone, infection, urinary obstruction, vascular emergency, pregnancy complication, or another cause must be considered; not approved for real-patient or production use.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the patient's exact age, where is the pain, when did it start, was onset sudden, and does it travel to the abdomen, groin, shoulder, or testicle?
2. [YES_NO] Is the patient confused, difficult to wake, fainting or collapsed, severely short of breath, cold or clammy, or rapidly worsening?
3. [YES_NO] Is there fever, shaking chills, vomiting, inability to keep fluids down, painful urination, foul or cloudy urine, blood in urine, or inability to pass urine?
4. [YES_NO] Is the patient pregnant, recently pregnant, or possibly pregnant, and is there one-sided pelvic or abdominal pain, vaginal bleeding, shoulder-tip pain, fainting, or reduced fetal movement?
5. [YES_NO] Is there a known abdominal aortic aneurysm, solitary kidney, kidney transplant, significant kidney disease, urinary abnormality, diabetes, immune suppression, recent urinary procedure, or anticoagulant use?
6. [YES_NO] Is the patient a child, unable to communicate symptoms reliably, or experiencing a first episode that has not been assessed in person?

Triage and redirect questions

1. Redirect → Back Pain

Is the pain centered in the spine or back muscles, without urinary or kidney-area symptoms?

Primary spinal or muscular back pain belongs in the Back Pain pathway.

1. Disposition level 100

Is there collapse, confusion, reduced responsiveness, severe breathing difficulty, cold or clammy skin, or rapid deterioration; sudden severe abdominal, back, or flank pain especially with a known aneurysm or pulsating abdominal mass; complete inability to pass urine with severe pain or illness; or pregnancy/possible pregnancy with severe one-sided abdominal or pelvic pain, vaginal bleeding, shoulder-tip pain, fainting, or collapse?

These features may indicate shock, ruptured abdominal aortic aneurysm, complete urinary obstruction, or ruptured ectopic pregnancy. NHS identifies sudden severe abdominal or back pain as an AAA emergency, and RCOG identifies severe pain or collapse as possible ruptured ectopic pregnancy.

1. Disposition level 90

After immediate emergency features are excluded, is there fever or shaking chills, feeling very unwell, severe or uncontrolled pain, vomiting or dehydration, blood in urine, painful or foul-smelling urine, reduced urine, inability to pass urine, a solitary or transplanted kidney, significant renal or urinary disease, diabetes or immune suppression; or is the patient pregnant, recently pregnant, or a child?

An infected obstructed kidney and sepsis require urgent hospital care. NICE recommends hospital or specialist assessment when pyelonephritis is accompanied by serious illness, pregnancy, dehydration, structural urinary abnormality, diabetes, immune suppression, or pediatric age.

1. Disposition level 70

With all emergency and higher-risk features excluded, is this new or recurrent flank pain that has not yet been assessed in person during this episode?

A kidney stone cannot be confirmed by telephone. Flank pain may require examination, urinalysis, renal function and infection tests, pregnancy testing where relevant, and age- and pregnancy-appropriate imaging before home management is judged safe.

Advice

Emergency flank or back pain â€” call Qatar 999

- Call Qatar 999 immediately and follow the operator's instructions. Keep the patient resting in the safest comfortable position, observed, and warm. Do not allow the patient to drive, walk unassisted, eat, drink large volumes, or take new medicine while a vascular emergency, ectopic pregnancy, shock, or complete obstruction is possible. If unresponsive and not breathing normally, start age-appropriate CPR as directed. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: collapse or reduced responsiveness; worsening pain, bleeding, breathing difficulty, or shock.

High-risk flank pain â€” same-day hospital assessment

- Arrange immediate same-day in-person assessment through the Qatar hospital or maternity destination approved for this UAT environment. Do not delay for a teleconsult. The patient may need urine and blood tests, renal-function and infection assessment, and imaging. Do not use leftover antibiotics or start, stop, or change medicine. A clinician or pharmacist must select analgesia after considering age, pregnancy, kidney function, allergy, bleeding risk, other medicines, and dehydration. Do not force fluids if the patient is vomiting, unable to urinate, or may have obstruction.
- Call Back If: fever, shaking, confusion, or worsening illness; inability to urinate; increasing pain, vomiting, bleeding, or fainting.

Undiagnosed flank pain â€” in-person assessment

- Arrange prompt same-day in-person assessment using the Qatar route approved for this UAT environment. A clinician may need urinalysis, blood tests, pregnancy testing where relevant, and imaging; suspected renal colic in pregnancy generally requires ultrasound rather than CT as the initial imaging approach. Until assessed, use

only medicine already confirmed as appropriate for this patient and do not assume the pain is a kidney stone or force excessive fluid intake.

- Call Back If: pain worsens or becomes sudden and severe; fever, vomiting, bleeding, reduced urine, fainting, or pregnancy-related symptoms.

First aid

- Call Qatar 999 immediately and follow the operator's instructions. Keep the patient resting in the safest comfortable position, observed, and warm. Do not allow the patient to drive, walk unassisted, eat, drink large volumes, or take new medicine while a vascular emergency, ectopic pregnancy, shock, or complete obstruction is possible. If unresponsive and not breathing normally, start age-appropriate CPR as directed. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 25 July 2026)
- NICE, "Pyelonephritis (acute): antimicrobial prescribing", NG111, <https://www.nice.org.uk/guidance/ng111/chapter/recommendations>
- NICE, "Renal and ureteric stones", QS195, <https://www.nice.org.uk/guidance/qs195>
- NICE, "Suspected sepsis in pregnant or recently pregnant people", NG255, 2025, <https://www.nice.org.uk/guidance/ng255>
- Royal College of Obstetricians and Gynaecologists, "Ectopic pregnancy", <https://www.rcog.org.uk/for-the-public/browse-our-patient-information/ectopic-pregnancy/>
- NHS.UK, "Abdominal aortic aneurysm", <https://www.nhs.uk/conditions/abdominal-aortic-aneurysm/>
- NHS.UK, "Kidney stones & Symptoms", <https://www.nhs.uk/conditions/kidney-stones/symptoms/>

Provenance

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